



NAME : Mrs. DEEPIKA SHARMA
Age/Gender : 58 Y 0 M 0 D /F
Ref Doctor : SELF
Ref. Cust : TEZ HEALTH
Client Code : KAD393

UHID NO/Visit ID : KAD393.00000003/KAD393.3
Collected : 24/Jul/2026 05:59PM
Received : 24/Jul/2026 06:40PM
Reported : 24/Jul/2026 07:07PM
Barcode : H5837716

DRL - 72

Test Name	Result	Unit	Bio. Ref. Range	Method
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GLUCOSE - FASTING (FBS) , NAF PLASMA

Fasting Glucose	158.60	mg/dL	70 - 110	Hexokinase
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Ref. for Biological Reference Intervals: American Diabetic Association.

Interpretation Notes:-

A blood glucose test measures the glucose levels in your blood. Glucose is a type of sugar. It is your body's main source of energy. Symptoms of high blood glucose levels include increased thirst, more frequent urination, Blurred vision, Fatigue, Wounds that are slow to heal. Symptoms of low blood glucose levels include Anxiety, Sweating, Trembling, Hunger and Confusion. Blood glucose test is required to check certain risk factors for diabetes. These include Being overweight, Lack of exercise, Family member with diabetes, High blood pressure, Heart disease.

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GLYCOSYLATED HAEMOGLOBIN (GHB/HBA1C) , WHOLE BLOOD EDTA

Glycosylated Haemoglobin HbA1c	7.3	%	Normal: < 5.6 Pre-Diabetes: 5.7 - 6.4 Diabetes: > 6.5	HPLC
Approximate Mean Plasma Glucose	162.81			Calculated

Ref.for Biological Reference Intervals: American Diabetes Association.

Interpretation Notes:-

The A1C test measures your average blood glucose for the past 2 to 3 months. Diabetes is diagnosed at an A1C of greater than or equal to 6.5%.

Therapeutic goals for glycemic control (ADA):

-Adults:

- ◆ Goal of therapy: < 7.0% HbA1c
- ◆ Action suggested: > 8.0% HbA1c

-Pediatric patients:

- ◆ Toddlers and preschoolers: < 8.5% (but > 7.5%)
- ◆ School age (6-12 years): < 8%
- ◆ Adolescents and young adults (13-19 years): < 7.5%


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VITAMIN - B12 , SERUM

Vitamin - B12	139	pg/mL	211 - 911	CLIA
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Interpretation Notes:-

Vitamin B12 and folate are critical to normal DNA synthesis, which in turn affects erythrocyte maturation. Vitamin B12 is also necessary for myelin sheath formation and maintenance. The body uses its B12 stores very economically, reabsorbing vitamin B12 from the ileum and returning it to the liver so that very little is excreted. Clinical and laboratory findings for B12 deficiency include neurological abnormalities, decreased serum B12 levels, and increased excretion of methylmalonic acid. The impaired DNA synthesis associated with vitamin B12 deficiency causes macrocytic anemias. These anemias are characterized by abnormal maturation of erythrocyte precursors in the bone marrow, which results in the presence of megaloblasts and in decreased erythrocyte survival. Pernicious anemia is a macrocytic anemia caused by vitamin B12 deficiency that is due to lack of intrinsic factor. Low vitamin B12 intake, gastrectomy, diseases of the small intestine, malabsorption, and trans-cobalamin deficiency can also cause vitamin B12 deficiency.

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25 (OH) VITAMIN-D , SERUM

25 (OH) Vitamin-D	46.50	ng/mL	Deficiency: < 10.0 Insufficiency: 10 - 30 Sufficiency: 30 - 100 Toxicity: > 100	CLIA
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Interpretation Notes:-

Vitamin D deficiency (not enough vitamin D). These symptoms include

- Bone weakness & Softness
- Bone malformation (in children)
- Fractures

Higher risk for a vitamin D deficiency.

- Osteoporosis or other bone disorder
- Previous gastric bypass surgery
- Age; vitamin D deficiency is more common in older adults.
- Obesity
- Lack of exposure to sunlight
- Having a darker complexion
- Difficulty absorbing fat in your diet

In addition, breastfed babies may be at a higher risk if they aren't taking vitamin D supplements.


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COMPLETE BLOOD COUNT (CBC) , WHOLE BLOOD EDTA

Haemoglobin	10.4	g/dL	12 - 15	Spectrophotometry - (non cyanide)
RBC Count	4.08	Millions/cumm	3.8 - 4.8	Electrical Impedance
Total WBC Count	7550	Cells/cumm	4000 - 10000	Electrical Impedance
Platelet Count	204	10 ³ /μL	150 - 450	Electrical Impedance
Packed Cell Volume (PCV)	34.6	%	36 - 46	Calculated
Mean Corpuscular Volume (MCV)	84.8	fL	83 - 101	Calculated
Mean Corpuscular Hb. (MCH)	25.4	pg	27 - 32	Calculated
MCHC	30.0	g/dL	31.5 - 34.5	Calculated
RDW CV	15.6	%	11.6 - 14.0	Calculated
RDW SD	47.3	fL	39 - 46	Calculated
PDW	16.0	fL	11 - 22	Calculated
MPV	12.0	fL	7 - 11	Calculated

Differential Count by Flowcytometry/Microscopy

Neutrophils	63.4	%	40 - 80	Impedance/microscopy
Lymphocytes	28.4	%	20 - 40	Impedance/microscopy
Eosinophils	3.1	%	1 - 6	Impedance/microscopy
Monocytes	4.8	%	2 - 10	Impedance/microscopy
Basophils	0.3	%	0 - 2	Impedance/microscopy

Interpretation Notes:-

A complete blood count gives information regarding the cell types in person's blood and concentration of haemoglobin. Cells that circulate in the blood are generally divided into three types: RBC, WBC and platelets. Abnormally high or low counts may occur in physiological conditions and in diseased states and requires clinical correlation. Differential counts and RBC indices help in further understanding of the likely aetiology.

NOTE: This report has been generated by a fully automated analyser after counting thousands of cells and hence differential count may appear as decimalized numbers.


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ERYTHROCYTE SEDIMENTATION RATE (ESR) , EDTA

Erythrocyte Sedimentation Rate (ESR)	117	mm in 1 hr	≤19	
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Note: ESR is an acute phase reactant which indicates presence and intensity of an inflammatory process. ESR is elevated in a wide range of organic diseases. ESR is not a specific and diagnostic test for any disease. However, it is helpful in differentiating functional from organic disease. Extremely high levels are found in cases of malignancy, hematologic diseases, collagen disorders and renal diseases.

Reference: Bates I. Reference Ranges and Normal Values. In: Bain BJ, Bates I, Laffan MA, Lewis SM, editor. Dacie and Lewis Practical Haematology, 12th ed. China: Elsevier publishers; 2017.pg. 8-17.


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LIPID PROFILE , SERUM

Total Cholesterol	77.50	mg/dL	Desirable : < 200 Borderline High :200 - 239 High : ≥ 240	Cholesterol Oxidase Peroxidase (CHOD-PAP)
HDL Cholesterol	22.50	mg/dL	> 40	Homogenous Enzymatic Colorimetric
Total Triglycerides	118.00	mg/dL	Desirable Level : < 150 Borderline : 150 - 199 High : 200 - 499 Very High : 500	GPO-POD
VLDL Cholesterol	23.60	mg/dL	≤ 30	Calculated
LDL Cholesterol	31.40	mg/dL	Optimal: < 100 Above Optimal: 100 - 129 Borderline: 130 - 159 High: 160 - 189 Very High: ≥ 190	Calculation
Non - HDL Cholesterol	55.00	mg/dL	≤ 130	Calculated
Chol / HDL Ratio	3.44		Low Risk: 3.3 - 4.4 Average Risk: 4.5 - 7.1 Moderate Risk: 7.2 - 11.0 High Risk : > 11.0	Calculated
TGL / HDL Ratio	5.2		Optimal: < 2.5 Mild to moderate risk: 2.5 - 5.0 High Risk: > 5.0	Calculated
HDL / LDL Ratio	0.72			Calculated
LDL / HDL Ratio	1.40		Desirable: 0.5 - 3.0 Borderline Risk: 3.0 - 6.0 High Risk: > 6.0	Calculated

Interpretation Notes:-

Lipid profile measures the amount of cholesterol and fats called triglycerides in the blood. These measurements give the doctor a quick snapshot of what's going on in blood. Cholesterol and triglycerides in the blood can clog arteries, making you more likely to develop heart disease.


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LIVER FUNCTION TEST (LFT) , SERUM

Total Bilirubin	0.27	mg/dL	0.3 - 1.2	Jendrassik-Grof
Direct Bilirubin	0.05	mg/dL	< 0.30 Neonates: < 0.6	Diazo
Indirect Bilirubin	0.23	mg/dL	0.3 - 1.0	Calculated
SGOT / AST	15.80	U/L	< 40	IFCC
SGPT / ALT	22.60	U/L	0 - 41	IFCC without pyridoxal phosphate activation
SGOT (AST) : SGPT (ALT) Ratio	0.70	Ratio	Refer Interpretation Notes	Calculated
Alkaline Phosphatase	109.00	U/L	35 - 104	IFCC, PNPP AMP Buffer
Total Protein	7.36	g/dL	6.4 - 8.3 Newborns: 4.6 - 7.0 Premature: 3.6 - 6.0 Umbilical cord: 4.8 - 8.0	Biuret
Albumin	3.97	g/dL	3.5 - 5.2	Bromocresol green
Globulin	3.39	g/dL	2.5 - 4.5	Calculated
A/G Ratio	1.17	Ratio	1.0 - 2.1	Calculated
Gamma Glutamyl Transferase (GGT)	7.00	U/L	Men: 8 - 61 Women : 5 - 36	Enzymatic Colorimetric

Interpretation Notes:-

The most common liver tests include:

Liver enzymes test: Your liver enzymes include alkaline phosphatase (ALP), alanine transaminase (ALT), aspartate aminotransferase (AST) and gamma-glutamyl transferase (GGT). These are elevated when there's liver injury.

Total protein test: A total protein test measures levels of protein in your blood. Your liver makes protein, and low protein levels may indicate that your liver isn't functioning optimally.

Bilirubin test: Bilirubin is a waste product that liver deposits in bile. Possible diagnoses may include: Fatty liver disease, Toxic hepatitis, Autoimmune hepatitis, Viral hepatitis (A, B or C), Hemochromatosis, Wilson's disease, Alpha-1 antitrypsin deficiency, Primary biliary cholangitis (PBC), Cirrhosis. Liver cancer.

SGOT (AST) : SGPT (ALT) Ratio: A normal AST:ALT ratio should be <1. In patients with alcoholic liver disease, the AST:ALT ratio is >1 in 92% of patients, and >2 in 70%. AST:ALT scores >2 are, therefore, strongly suggestive of alcoholic liver disease and scores <1 more suggestive of NAFLD/NASH.


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THYROID PROFILE (TFT) -I, SERUM

Triiodothyronine Total (TT3)	85.10	ng/dL	81 - 178	CLIA
Thyroxine (TT4)	6.60	µg/dL	4.5 - 10.9	CLIA
Thyroid Stimulating Hormone (TSH)	2.08	µIU/mL	0.4 - 5.5	CLIA

Interpretation Notes:-

TSH	Total T4	Total T3	Disease
Normal	Normal	Normal	None
Low	High	High	Hyperthyroidism
Low	Normal	Normal	Subclinical Hyperthyroidism
Low	Normal	High	T3 Toxicosis
Low	High	Normal	Thyroiditis, T4 ingestion, hyperthyroidism in the elderly or with comorbid illness
Low	Low	Low	Euthyroid sick syndrome; central hypothyroidism
High	Normal	Normal	Subclinical hypothyroidism; recovery from euthyroid sick syndrome

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4 AM and at a minimum between 6 - 10 PM. The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
- Unbound fraction (Free, T4/Free, T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration.
- Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.


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RENAL FUNCTION TEST (RFT) , SERUM

Urea	46.31	mg/dL	16.6 - 48.5	Urease
Creatinine	0.78	mg/dL	Females: 0.50 - 0.90	Modified Jaffe Kinetic
BUN	21.64	mg/dL	7 - 18	Calculated
BUN/Cr Ratio	27.74			Calculated
Uric Acid	4.30	mg/dL	Male: 3.4 - 7.0 Female: 2.4 - 5.7	Uricase
Sodium	132.00	mmol/L	135 - 150	ISE
Potassium	4.90	mmol/L	3.5 - 5.0	ISE
Chloride	101.80	mmol/L	94 - 110	ISE
Calcium	8.25	mg/dL	8.6 - 10.0	BAPTA

Interpretation Notes:-

Kidney/Renal function tests (KFT/RFT) are usually ordered when a patient has risk factors for kidney dysfunction such as hypertension, diabetes, cardiovascular disease, obesity, elevated cholesterol or a family history of kidney disease. It may also be ordered when someone has signs and symptoms of kidney disease, though in early stage often no noticeable symptoms are observed. Kidney panel is useful for general health screening; screening patients at risk of developing kidney disease; management of patients with known kidney disease.


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IRON PROFILE 2

TIBC	238.73	ug/dL	250-450	Ferene-S
Iron	45.13	µg/dL	37 - 145	FerroZine
Transferrin Saturation%	18.90		12 - 50	Calculation
Transferrin	191	mg/dL	200 - 360	Calculation
Unsaturated Iron Binding Capacity (UIBC)	194		112 - 347	Ferro Zine colorimetric

Interpretation Notes:-

Serum iron, total iron-binding capacity, and percent saturation are widely used for the diagnosis of iron deficiency. Symptoms of iron levels that are too low include:

- Pale skin
- Fatigue
- Weakness
- Dizziness
- Shortness of breath
- Rapid heart beat

Symptoms of iron levels that are too high can vary and tend to get worse over time. Symptoms may include:

- Joint pain
- Abdominal pain
- Lack of energy
- Weight loss

Printed On :25-Jul-2026 01:23 PM, Sample Processed at : DRL - TAMBARAM,

Result/s to Follow:

CUE - COMPLETE URINE ANALYSIS (AUTOMATED REAGENT STRIP METHOD AND MICROSCOPY)

*** End Of Report ***

Disclaimer:

1. It is presumed that the specimen belongs to the patient named or identified in the test request form. 2. Results pertain to the specimen submitted and all results are dependant on the quality of sample received in the laboratory. 3. The test results are for information and interpretation of the referring doctor or other medical professional who understands reporting units, reference ranges and limitations of terminologies.4. Laboratory investigations help in arriving at a diagnosis and should be

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